

QuriioHQ — Marketing Brief

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Build the ad around one idea: QuriioHQ ends the crowded OPD corridor by telling every patient exactly where they stand in the queue — on WhatsApp, in their own language, with no app. The ward (IPD) module then carries the same patient from admission to a clean, itemised discharge bill.

This brief describes what is built in the product today. Two things in the original request do not match the product, and the ad must not use them: see *Claims guardrails* below.

What it is and who it is for

QuriioHQ is cloud software for small and medium hospitals and clinics in rural and semi-urban India. A typical customer sees 150–200 outpatients a day, works in Marathi, Hindi and English, and has patients on budget Android phones and patchy internet.

The core problem it solves is **uncertainty, not booking**. A patient does not know if their turn is 20 minutes or 3 hours away, so they never leave the building. By 10am the corridor is full, reception is answering "how much longer?" all morning, and consultations get interrupted.

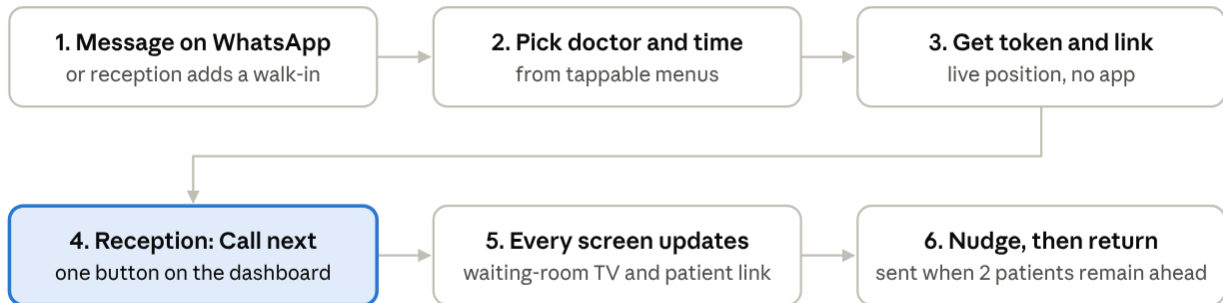
Audience	What they care about	Lead with
Hospital owner (usually a doctor)	A calm OPD, honest numbers on how the day ran, no new IT headache	Calm corridor, one dashboard, reports
Hospital administrator / manager	Less desk chaos, clean billing, staff accountability	One-button queue, audit trail, gap-free bills
Doctors	Fewer interruptions, faster prescriptions	Own queue on their phone, repeat-last prescription
Solo doctors and small clinics	Works without a receptionist	Doctor advances the queue from their own phone

The main reader is **a doctor who owns a hospital**. They are not shopping for software; they are tired of the crowd outside their door. Write to that person.

How it works

The whole product fits in six steps, and this is the story the ad should tell. A doctor should understand QuriioHQ from this flow alone.

One Call next press updates every screen and nudges the patient back



patient journey · 6 steps

Step 4 is the hero moment: one press at the desk changes what the patient, the TV and the doctor all see. Between steps 3 and 6 the patient is free to wait anywhere.

Features by module

Only the **Ready** rows are safe to advertise as available today. *Built, not live* means coded and tested but not yet switched on for a hospital; ask the founder before featuring it.

Module	What it does	Status (10 Oct 2026)
WhatsApp booking	Patient messages the hospital, picks doctor and time from tappable menus, gets a token number and a live queue link	Ready — runs end-to-end on Meta's live WhatsApp API
Live queue page	Patient sees their position on a web link: no app, no login, Marathi / Hindi / English	Ready
Reception dashboard	One <i>Call next</i> button; skip, hold, recall, priority insert, walk-in registration	Ready
"Come back now" nudge	Patient is messaged automatically when two patients remain ahead	Ready

Module	What it does	Status (10 Oct 2026)
Waiting-room TV display	"Now serving" numbers readable across the room, with audio	Ready
Solo-doctor mode	Doctor runs the queue from their own phone, no receptionist needed	Ready
Reports and audit	Patients seen, waits, consultation times; who moved which token and when	Ready
OPD billing	Consultation fee per doctor, one-tap Paid / Unpaid, "Seen today" list	Ready
Prescriptions	Medicine catalogue, diagnosis and prescription on one card, repeat last prescription, printable A5 prescription	Built, not live
IPD (ward)	Shift to IPD in one click, beds and wards, nurse bedside entries on a phone (works offline), nightly bed charges, itemised discharge bill, family running-bill link in 3 languages, Undo on every action	Built, not live
Ward charts	Temperature / pulse / BP chart, medicine-given record, time-critical dose alerts	In pilot development — do not advertise
Lab-lite	Basic lab orders and results	Planned for 2027 — do not advertise

Benefits by audience

The three benefits to lead with — efficiency, patient experience and cost control — map to concrete product behaviour, not measured results. Describe *how* it works; no percentages (see guardrails).

Benefit	For whom	What makes it true
A calm corridor	Owner, patients	Patients know their position, so they wait outside, at a tea stall or at home, and return when nudged
Fewer interruptions	Doctors	Nobody needs to ask "how much longer?" — the answer is on the patient's phone and the TV
One button at the desk	Reception	<i>Call next</i> replaces the paper register and shouting names; walk-ins added in seconds
Honest numbers	Owner	Reports show how the day actually ran: patients seen, median wait, median consultation
Accountability	Owner, administrator	Audit log of every queue move; in IPD, every bedside entry is signed by the nurse's own login
No lost charges	Owner, billing desk	In IPD, every bedside entry becomes a bill line instantly; bed charges added nightly; bill numbers have no gaps
Respect for the patient	Patients and families	No app to install, their own language, a running-bill link for the family
Predictable cost	Owner	Monthly plan sized to patient volume; no hardware beyond a phone and an optional TV
Quick start	Reception	Designed to be learned in about 15 minutes; sits beside existing hospital software rather than replacing it

Message pillars and ad copy starters

Use four pillars, in this order of priority. Headlines name the doctor's problem, not the feature.

- 1. Empty the corridor.** Patients wait where they like and come back on time.
 - "Your OPD, without the crowd outside your door."
 - "They know their number. You get your corridor back."
- 2. No app. Their language.** WhatsApp and a link, in Marathi, Hindi or English.

- "If your patients use WhatsApp, they can already use QuriioHQ."

3. **One button for reception.** *Call next* replaces the register and the shouting.

- "One button. No paper register. No double entry."

4. **OPD to discharge, one system.** Admission, bedside entries and an itemised bill that misses nothing.

- "Every bedside entry becomes a bill line. Nothing slips through." (*only once IPD is live — see open questions*)

Proof to show instead of statistics: real screenshots of the patient's phone ("3 patients ahead of you"), the reception dashboard and the waiting-room TV. A short screen recording of one *Call next* press updating all three is the strongest single visual.

Visual tone (from the existing landing-page brief): calm, clinical, trustworthy; teal as an accent; real interface mockups, no stock photos of doctors or patients.

Claims guardrails

A single false claim ends a healthcare sale the moment a doctor asks "which hospital said that?". These rules come from the product's own landing-page brief and from what is actually built.

Do not say	Why	Say instead
"Reduces wait times by 30%" or any other percentage	The 30% and "saved thousands" examples are from <i>other</i> hospitals' projects. QuriioHQ has no measured results yet — it has not run in a live waiting room	Describe how it works: "patients see their exact position and come back when called"
Testimonials, hospital logos, "trusted by N hospitals", star ratings	There are no paying customers yet; one pilot hospital is in progress	Nothing — cut the section until a pilot hospital agrees to be named
"Real-time inventory management"	Not built. There is a medicine and charge price list, not stock tracking	Leave out

Do not say	Why	Say instead
"Integrated electronic health records (EHR)"	Not built as an EHR. Prescriptions and ward records are being added, but it is not a full EHR and does not integrate with other systems	"Works alongside your existing hospital software"
"Automated scheduling" or "staff scheduling"	Patients book a doctor and time on WhatsApp, but there is no staff rostering	"Book on WhatsApp"
"Never stores medical data"	True for the OPD queue alone, but the newer prescription and IPD modules do store clinical records	"Patient data is isolated per hospital and every access is logged"
"Trained in 15 minutes" as a fact	It is a design goal, not yet measured	"Built to be learned in one short session"
Any medical or clinical-outcome claim	It is an operations tool, not a clinical device	Talk about flow, time and billing
"Data stored in India"	Patient data is currently hosted in Singapore; a move to India is planned	Leave out until the move is done

Open questions for the founder

Answer these before the creative is briefed out.

- ☒ **Brand name.** ~~The app uses *QurioHQ*; internal docs also say *QueueCare* and *Qurio*. Confirm the spelling and whether "HQ" is part of the public name.~~
- ☐ **Show IPD in the ad?** It is built and tested but not yet live at a hospital. Recommendation: OPD-only for the first ad; add IPD once the pilot hospital is using it.
- ☐ **Show prices?** The plan list in the code runs from about ₹1,999/month (Solo) to ₹12,999/month (Multi-branch), sized by patients per day, with annual billing at 10 months' price. Confirm these are final before printing any number. Recommendation: say "plans sized to your patient volume" and keep numbers for the demo call.
- ☐ **Pilot hospital as a reference.** Can the pilot hospital be named once results exist? Until then, no names.

- ☐ **Call to action.** The product already has a 5-field demo-request form. Recommendation: every ad points to "Book a free demo".